



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان سامى فهمى محمود

العنوان : 5 ح الشيخ المنوفى -
الزير المعلق

الحي / المدينة : عابدين

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 01-10-2026

رقم البطاقة : 27505240103245 رقم التليفون : 01147783483

العمر (تاريخ الميلاد) : 24-05-1975 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان سامى فهمى محمود National ID: 27505240103245	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name : الأستاذ / حنان سامي فهمي محمود	Request No. : 13003594
Patient No. : 27505240103245	Request Date : 24/11/2025
Path No. : 3640/25	Verification Date : 02/12/2025
Sex : Female	Financial Category : جونتاف روسي - مياره
Age : 50 Y	Department : Out Patient
Referring Doc. :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast fibrocystic changes with two amorphous clusters of microcalcifications (BIRADS 4b), in patient with history of right mastectomy on hormonal therapy.

Nature of Specimen: Stereotactic guided Tru-cut biopsy, collected on 24/11/2025, at 10:00 am.

Gross Examination:

- Multiple fragmented gray white tissue cores, the longest is 1 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of breast tissue showing diffuse stromal fibrosis with focal adenosis & irregular cystically dilated ducts. The ducts are lined by inner columnar and outer myoepithelial cells. There is focal sclerosing adenosis & multiple foci of microcalcification.

No evidence of malignancy in section examined.

⇒ No DCIS or malignancy

Diagnosis:

Left breast lesion, Tru-cut biopsy:

Fibrocystic changes with focal sclerosing adenosis & multiple foci of microcalcification.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Duaa Mohamed.

Follow up
after 6 months

does it benign or
suspicious for
DCIS
دكتور

Verified by :

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Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: حنان سامى فهمى		
MR. : 1394		
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is currently receiving adjuvant tamoxifen therapy, initiated in January 2025, with a planned duration extending through February 2025 for routine transvaginal ultrasound surveillance. The regimen includes tamoxifen 20 mg daily, supplemented with calcium 1 mg, 1 × 90 mg, and one dose of alpha-agonist as noted. In the most recent assessment, the patient’s sonographic evaluation in April 2026 revealed a BIRADS 2 finding, and a transvaginal ultrasound in June 2026 identified a uterine fibroid measuring 22 × 16 mm with a myometrial thickness of 7 mm. Tamoxifen therapy continues at 20 mg daily, with supportive care planned for an additional three months. The next follow-up is scheduled for 23 September 2026.		
تحريرا في ,		
	التاريخ :	20-09-2026
	الوقت :	
	الطبيب المعالج :	ايه مدحت بدوي محمد
يعتمد ,		

Patient Name: Hanah Sami Fahmy Mahmoud	Date: 14/09/2025
Exam: MAMMOGRAPHY BOTH	Age: 50 Year(s)
Medical ID: 10000000000000000000	Pat ID: 94373

**Left Digital soft tissue mammography with high resolution
sonography revealed:**

Clinical summary: Post right MRM & ALAND, on hormonal therapy, routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- Two mildly suspicious clusters of amorphous microcalcifications are noted, one retroareolar and one seen LIQ, middle third, 7 cm from nipple.
- Two UOQ circumscribed oval equal density mass lesion with obscured margins.
- Scattered punctate probably benign macrocalcifications.
- No architectural distortion.
- Ultrasound demonstrated no solid lesions. Few scattered cysts, the majority are simple yet two UOQ complicated ones are noted in terms of having thick wall and turbid content, largest is middle third at 2 o'clock measuring 12 mm in greatest dimension.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

Left breast (BIRADS 4b): fibrocystic changes with two amorphous clusters of microcalcifications.

Clear right mastectomy bed and axilla.

Recommendation:-

For Stereotactic biopsy.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
AM 11:59 2025/09/14



Ministry of Health and Family Welfare
Government of India

Ministry of Health and Family Welfare
Government of India



MDT Form

3. MDTs (MDT) -
Example

51712935
Dated

13/02/2024

1. Patient ID

2. Name

Sex

Patient Age 50 Y



Reason of MDT

Date of MDT

13/02/2024

Family history

Phone

011-1212345

BP

Mental status

AKS Score

GROUP

Comorbidity

Steropausal status

Not at stage

Clinical data

Breast Imaging

Ultrasound mass 5x2x1 cm, microcalcifications in the 20s.

Mets Workup :
PAUS Fatty liver
CE cap free

DX
10C

ER
8/8

PR
8/8

Her 2 neg

SISH

Ki67
5

1
2

4
0

M
D

Sonomammography

PATIENT HISTORY

UPERONT RT SIA SLNB: free margin
LN 0/8

MDT OUTCOME & PLAN

1. ADJUVANT TAXOTERE ENDOXAN X4
2. ADJUVANT TAM THEN SWITCH

MEDICAL ONCOLOGY

SURGERY

PSYCHIATRY

SIGNATURE

RADIOLOGY

PATHOLOGY

Medication

Patient Name : الأستاذة / حنان ماضي محمد
Patient No. : 27505240103245
Path No. : 119/25
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13002949
Request Date : 11/01/2025
Verification Date : 06/02/2025
Financial Category : اورام دار السلام - مبادئ
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Up-Front.
Nature Specimen: Right simple mastectomy and SLNs.

Gross Examination:

- * Procedure: Right simple mastectomy specimen measured 18.5x14x3 cm with covering skin ellipse measured 17x9 cm.
 - * Tumor extent: A firm greyish white retro-aerolar mass.
 - * Size: 3.5x3x1.5 cm
 - * Skin involvement: Unremarkable grossly.
 - * Distance from margins: 5 cm away from deep margin and 1 cm away from the skin.
 - * Sentinel lymph nodes number and size: Two fibrofatty tissue pieces measured 6x4x1 cm and 3x3x1 cm, entangling 8 nodules, up to 2x1x0.8 cm (5 sentinel lymph nodes and 3 small were left for paraffin).
 - * Additional findings:
There was a rubbery grey white area measured 4.5x2x1.5 cm away 1.5 cm from the mass.
 - * Frozen results:
Free SLNs (0/5) and 3 small lymph nodes were left for paraffin.
 - * Designation:
1-4: Mass.
5: Area in between.
6&7: Grey white area.
8: Deep margin.
9: Nipple.
10&11: SLNs.
- Dissected by: Dr. Menan Hossam.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.

Verified by :
Dr. Foka Modhat

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Page 1 of 3

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Room No. (1) - First Floor Building No. 987- Nile Corakhe - Mier El Qadema - Cairo, Egypt

Patient Name : الأستاذة / ديار ياسين الهادي محمود
Patient No. : 27505240103245
Path No. : 119/25
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13002949
Request Date : 11/01/2025
Verification Date : 06/02/2025
Financial Category : أورام نادر الثدي - مفاخرة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- Angioinvasion: Not detected.
- Ductal carcinoma in situ: One duct showed apocrine DCIS.
- Skin involvement: Free.
- Lymph node involvement: Free SLNs in (0/8) lymph nodes.
- Resection Margins: Free deep surgical resection margin.
- Microcalcifications: Positive.
- Additional pathologic findings: Usual ductal hyperplasia, fibrocystic changes with papillary apocrine metaplasia.

Diagnosis:

Right breast, simple mastectomy and SLNs Biopsy:

- Invasive duct carcinoma, grade II with one duct showing apocrine DCIS and microcalcifications.
- Rest of breast showed usual ductal hyperplasia, fibrocystic changes with papillary apocrine metaplasia.
- Free deep surgical resection margin.
- Free skin.
- Free SLNs (0/8).
- Stage: pT2, pN0.

Signature

Prof. Dr. Gamal Abdelattay
Dr. Menna Nabil
Dr. Yasmine Essam

Verified by :
Dr. Toka Medhat

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / خديجة سامي إسماعيل محمد

Patient No. : 27505240103245

Path No. : 119/25

Sex : Female

Age : 49 Y

Referring Doc :

Request No. : 13002949

Request Date : 11/01/2025

Verification Date : 06/02/2025

Financial Category : اورام خارج المستشفى - مياومة

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Verified by :

Dr.Toka Medhat

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name : **المرشدة / هديل سامي الهادي محمود**
Patient No. : **27505240103245**
Path No. : **4616/24**
Sex : **Female**
Age : **49 Y**
Referring Doc :

Request No. : **13009949**
Request Date : **25/11/2024**
Verification Date : **11/12/2024**
Financial Category : **أمر نموذجي - مستورد**
Department : **Out Patient**
Registered by : **Mrs. Mayada Hegazy**

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 25/11/2024

Gross Examination:

- 3 grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II
- Ductal carcinoma in situ: Not detected
- Perineural invasion: Not detected
- Angiolysis: Positive
- Microcalcifications: Present
- IPT (Positive control showed appropriate reactivity).
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in < 10% of tumor cells (score +1).
- Ki67: 5% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy
Invasive duct carcinoma, Grade II
IPT:

- ER: Positive (score 8/8).

Verified by:
Dr. Heba Hasan

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Room No. (4) - First Floor-Building No. 987 Nile Corniche - Mir El Qadima - Cairo, Egypt

Patient Name : الأ.د. هبة / هادي هادي
Patient No. : 27505240103245
Path No. : 4616/24
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13009949
Request Date : 25/11/2024
Verification Date : 11/12/2024
Financial Category : مريض - مريض
Department : Out Patient
Registered by : Mrs Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 25/11/2024

Gross Examination:

- 3 grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II
- * Ductal carcinoma in situ: Not detected
- * Perineural invasion: Not detected
- * Angiolymphatic invasion: Positive
- * Microcalcifications: Present
- * IPT (Positive control showed appropriate reactivity)
- ER Strong: nuclear reaction in 80% of tumor cells (score 8/8).
- PR Strong: nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in < 10% of tumor cells (score +1)
- Ki67: 5% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy
Invasive duct carcinoma, Grade II
IPT:

- ER: Positive (score 8/8).

Verified by :
Dr. Heba Hasan

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Page 1 of 3

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مختبر الأمراض النسائية - مبنى رقم (1) - شارع (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mer El Qadima - Cairo, Egypt



Patient Name: Hanan Sami Fahmy Mahmoud	Date: 12/12/2024
Exam: C.T ABDOMEN & PELVIS	Age: 49 Year(s)
Medical ID: 4364/2024	Pat ID: 94373

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- > Bilateral breast asymmetrical dense parenchymas are seen .
- > Both lungs show mosaic attenuation with areas of air trapping and bilateral small pneumatocele.
- > Few subcentimetric pericarinal and prevascular lymph nodes, The largest is the former and measures about 9x9mm.
- > No evidence of pleural effusion, pleural thickening or masses seen.
- > Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- > Normal CT appearance of the thoracic aorta and the great vessels.
- > Bilateral small fibroelastoma dorsi are seen.
- > Left thyroid iso to hypodense nodule is seen for US correlation.

II Abdomen & pelvis:

- > The liver is of average size with homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- > Cholecystectomy clips are noted with subsequent mild dilatation of CBD and CHD on non contrast basis CT study.
- > No intra-hepatic biliary radicles dilatation.
- > Normal size & parenchymal texture of the spleen with no focal lesions seen, apart from small splenule is noted.
- > Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- > Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- > Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- > Left adenal likely ovarian cyst is noted measures about 26x27 mm likely functional cyst.
- > No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- No definite worrisome lytic or sclerotic focal lesions.
- Scoliotic list with the convexity to the right.



Patient Name: Hana Sami Fahmy Mahmoud	Date: 12/12/2024
Exam: C.T ABDOMEN & PELVIS	Age: 49 Year(s)
Medical ID: 4364/2024	Pat. ID: 94373

Opinion:

- Bilateral mosaic lung attenuation likely small airway disease.
- Left adenexal likely functional ovarian cyst.
- No evidence of pulmonary, hepatic or osseous metastatic deposits.

DR/Male Khattab, MSc.

Dr. Walid Mohamed, M.D.



Patient Name: Hanan Sami Fahmy Mahmoud	Date: 19/11/2024
Exam: ultrasound BREAST (BOTH SIDE)	Age: 49 Year(s)
Medical ID: 27505240103245	Pt. ID: 94373

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

- Bilateral heterogenous fibroglandular parenchyma which may obscure small focal lesions (ACR C)
- Right Breast:
 - Retro-areolar irregular hyperdense mass with spiculated outlines, studded with foci of calcifications. Complementary US showing this lesion as an irregular hypoechoic mass with spiculated margins and surrounding desmoplastic reaction extending from 10 to 12 o'clock. It measures 50x30mm reaching overlying skin and nipple.
 - UOQ regional area of amorphous calcification over an area measuring 54x24x19mm not corresponding to significant findings by US
 - An oval well-circumscribed hypoechoic lesion with lobulated outline is seen at 2O'clock (appears isodense on Mammography). It measures about 12x8mm
 - Nonspecific level I axillary LNs
 - No pathological enlarged levels II, III axillary or supraclavicular lymph nodes.
- Left Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary L.N.

Opinion:

- Right breast malignant mass and malignant UOQ microcalcifications (BIRADS 5) with right 2 o'clock indeterminate lesion for histopathological examination of both masses.
- Normal sonomammography of left breast (BIRADS I)

Best regards,
Dr Reem Mahmoud
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCL, Cairo University